

PROFESSIONAL WRITING

Chapter 6

Introduction

- Professional writing is certainly not limited to journal articles or books.
- It includes
 - writing evaluations of medications for consideration on a hospital formulary,
 - preparing written policies and procedures for the preparation of an intravenous admixture,
 - reporting the results of the latest sales to the home office,
 - writing on a professional website,
 - responding to a drug information question via e-mail,
 - preparing a written evaluation of a technician or clerk,
 - writing in a chart,
 - writing a term paper for a class,
 - preparing slides or posters for a presentation,
 - writing a letter of recommendation, and many other things

Introduction

- So whether the objective is to write a book or to type a label, a pharmacist must know how to write professionally.
- Generally **four larger purposes** for the existence of written material.
 - inform,
 - instruct,
 - persuade, or
 - entertain.
- The *first three items* are those usually considered in professional writing

Drug and Therapeutics Committee (DTC)

Chapter 8

Drug and Therapeutic Committee (DTC)

- The pharmacy (Drug) and therapeutics committee is an **advisory group** of the medical staff and serves as the organizational line of communication between the medical staff and pharmacy department.
- It is a **policy making body** to the medical staff and the administration of the hospital on matters related to the therapeutic use of drugs.
- Each hospital should establish DTC to promote the safe, rational and cost-effective use of medicines.

Membership of DTC

- DTC should be multidisciplinary including (as minimum):
 - Chief Clinical Officer, or equivalent (Chairperson)
 - Senior Pharmacist (Secretary)
 - A representative from each Case Team
 - A representative clinician from each major specialty (internal medicine, surgery, pediatrics, obstetrics and gynecology etc)
 - Senior nurse representative
 - Representative from hospital finance department
 - Representative from other services as deemed necessary

DTC

- Other non-voting, non-executive members may be invited to attend DTC meetings to discuss issues that require their particular expertise.
- **Sub-committees of the DTC** may be formed to address specific issues as the need arises (for example a policy on the use of antimicrobials etc).

DTC

Goals and objectives of the DTC

- The goal of a DTC is to ensure that pts are provided with the best possible cost effective and quality of care through determining what medicines will be available, at what cost, and how they will be used.

Objectives of DTC

- To develop and implement an efficient and cost-effective **formulary system**
- To ensure that only efficacious, safe, cost-effective and good quality medicines are used
- To ensure the best possible drug safety through monitoring, evaluating and thereby preventing, as far as possible, ADRs and MEs
- To develop and implement interventions to improve medicine use by prescribers, dispensers and pts

DTC Meetings

- The DTC should meet at a minimum **every two months**
- Minutes of all DTC meetings should be kept .
- The agenda, supplementary materials and minutes of the previous meeting should be prepared by the secretary and distributed to members for review in sufficient time before the meeting.
- All DTC recommendations should be disseminated to the medical staff and other concerned parties and authorities

Roles and responsibilities of the DTC

1. To develop and maintain the hospital formulary

- All hospitals should have a hospital formulary listing all pharmaceuticals that can be used in the facility.
- A participatory process should be followed to develop the formulary involving clinical, laboratory, diagnostic imaging and finance personnel.
- The selection of pharmaceuticals for the Formulary should be based on:
 - National drug list (List of Drugs for Ethiopia)
 - Federal Ministry of Health's **Essential Health Services** package,
 - STGs
 - Local pattern of disease

Roles and responsibilities of the DTC

- During preparation of the formulary emphasis should be placed on:
 - Drug description using generic names
 - Dosage form, strength and package size – in **basic units**
(Eg: Amoxicillin 500mg, capsule, 100 per pack)
- The formulary should be reviewed and updated at **least annually**.

Roles and responsibilities of the DTC

2. To develop standard clinical/treatment guidelines

- STGs promote rational drug use and provide a benchmark of optimum treatment for the monitoring & audit of drug use.
- DTC should implement national STGs for use in their own hospital.
- Hospitals should develop **their own STGs** for common clinical conditions that are not covered by national STGs.
- A participatory process should be followed.

Roles and responsibilities of the DTC

3. To develop policies and guidelines for managing formulary and non-formulary items

- Such policies and guidelines include:
- Policies/guidelines for the selection, quantification, procurement and distribution of pharmaceuticals.
- Policies/guidelines for generic substitution and therapeutic interchange.
- Policies/guidelines on the use of specific medications for example narcotics, chemotherapeutic agents, highly expensive medications, etc.

Roles and responsibilities of the DTC

4. To establish mechanisms to identify and address drug use problems

- The DTC should establish a policy and procedures for identifying and managing drug use problems including, as a minimum:
 - Monitoring ADRs, Prescription monitoring, Drug utilization monitoring, Antimicrobial prescribing and use
- **Strategies** should be established to correct problems identified.
- Such interventions may include:
 - Educational programs and in-service training,
 - Use of STGs and formularies, and
 - Prescribing restrictions, etc.

Roles and responsibilities of the DTC

5. To establish and oversee the DIC

- Each hospital should establish a drug information service that provides information and advice to health professionals, pts and the public.

Roles and responsibilities of the DTC

6. To develop an annual action plan

- The DTC should prepare an annual action plan, describing the main tasks to be undertaken in the year with corresponding budget requirements to cover areas such as trainings, printing etc.

Legal Responsibility for the Provision of Drug Information

Chapter 9

Introduction

- Legal obligations of the DI specialist remain unclear
- If you voluntarily offer and create a higher standard of careful practice, the public has a legal right to assume that pharmacists can and will consistently perform according to that standard (Willig)
- Professional standards of performance may be used by courts as an objective measuring tool for the standard of care
- Pharmacy profession is assuming an increased legal responsibility to provide DI in the daily practice of pharmaceutical care

Definitions

- **Ethics** is a set of moral principles and a code for behavior that govern an individual's actions with other individuals and within society.
- **Laws** are societal rules or regulations that are obligatory to observe. It is a set of principles enforced by the government and governed practice of healthcare professionals.

Basic Ethical and Legal Principles

- **Non-maleficence:** *“Do no Harm”*.
 - is the obligation to avoid or not inflict harm on others whether intentionally or unintentionally.
 - For example, if a pharmacist encounters a patient who is taking a harmful medication, the action that is guided by non-maleficence is to stop the patient from taking the medication
- **Beneficence:** *“Duty to promote good”*.
 - The principle of beneficence emphasizes the importance of weighing risks and benefits.

Basic Ethical and Legal Principles

- **Respect the patients-professional relationship.**
 - A moral rule often referring to respect for the health professional – patient relationship.
- **Justice: “fairness”.**
 - The principle of justice is concerned with the fair distribution of benefits and burdens.
 - Concerns of justice often arise when resources are scarce, or inequality is present. .

Basic Ethical and Legal Principles

- **Consent: “right to be informed”.**
 - A moral rule related to the principle of autonomy which states that the client has the right to be informed and freely choose a course of action. E. g. , informed consent to receive a therapy or a procedure.
- **Confidentiality: “right to give or refuse consent”.**
 - A moral rule also related to a principle of autonomy, which specifically addresses client’s right to give or refuse consent relative to release of privileged information.

Basic Ethical and Legal Principles

- **Privacy: “right to control own affairs”.**
 - The right of the individual to control his or her own affairs without interference from or knowledge of outside parties.
- **Veracity: “truth telling or honesty”.**
 - Related to the principle of autonomy is the principle of veracity, or truthfulness.
 - When informing and educating patients and caregivers, it is critical to provide truthful information even though such information may involve “bad news” or be perceived as harmful to the overall well-being of the patient.

Negligence

- Is a failure to take reasonable care to avoid causing injury or loss to another person.
- There are four steps in proving negligence

The *plaintiff* must prove that:

- there is a duty in the circumstances to take care (duty of care)
- the behaviour or inaction of the defendant in the circumstances did not meet the standard of care which a reasonable person would meet in the circumstances (*breach of duty*)
- the plaintiff has suffered injury or loss which a reasonable person in the circumstances could have been expected to foresee (*damage*)
- the damage was caused by the breach of duty (*causation*).

Negligence

- Elements of negligence (the four Ds)
 - Duty breached
 - Damages
 - Direct causation and
 - Defenses absent

Negligence

- Information negligence may occur because of
 - 1.Parameter negligence (failure to consult the correct source)
 - 2.Omission negligence (consulting the correct source but failure to locate the correct answers)

Negligence

- There are a number ways in which tort liability can attach to the provision of DI which includes
 - Incomplete information
 - Inappropriate quality information
 - Outdated information
 - Inappropriate analysis and
 - Inappropriate dissemination of information

Some liabilities

- **Comparative negligence** is the allocation of responsibility for damages incurred between the plaintiff and defendant, based on the relative negligence of the two
- **Concurrent negligence** is the wrongful acts or omissions of two or more persons acting independently, but causing the same injury
- **Vicarious liability** is the imputation of liability on one person for the actions of another
- **Respondeat superior** refers to the proposition that the employer is responsible for the negligent acts of its agents or employees

Legal and ethical issues related to DIS

- **Information resources**
 - Information resources shall be current and appropriate to the level of service provision and shall be documented whenever used in formulating responses.
- **Staff**
 - Staff shall be appropriately trained and supervised. Every effort shall be made to ensure maintenance of current awareness of the specialty.

Legal and ethical issues related to DIS

- **Query taking**

- Prior to answering queries, the identity of the enquirer and the reasons for the enquiry must be established.
- Patient enquires shall be referred to their local pharmacist or clinician if deemed appropriate.
- Drug information from the media or legal representatives shall be channeled through the health directorate.
- If a query is deemed unethical or illegal, the drug information pharmacist shall:
 - Refuse to answer the query or
 - Seek advice from the office of the Health Administrator, or the Solicitor on Council. However, all such queries shall be duly documented.

Legal and ethical issues related to DIS

- **Answering queries**

- DI pharmacists shall bear full responsibility for the quality of information or advice they provide.
- When necessary; queries shall be **referred to appropriate specialist for expert opinion.**
- Data extrapolated from **animal and in-vitro studies** must be stated clearly in the response as to the origins and limitations of the information.
- **Data from abstract** of an article must be stated clearly in the response as to the origins and limitations of the information.

Legal and ethical issues related to DIS

- **Answering queries...**
 - DI pharmacist shall have responsibility to their enquirers and their welfare. However, every effort must be made to ensure that the **relationship between patients and prescribers** is not compromised.
 - Responses shall be supported with reference and be made readily available on request.
 - Responses based on interpretation or evaluation of **limited available data** rather than substantial information must state so.
 - Information on ‘**non-approved**’ use of drugs shall be accompanied with a clear awareness of the prescriber’s responsibility in prescribing outside the approved indication.

Legal and ethical issues related to DIS

- Documentations
 - DI pharmacists shall ensure documentation of all queries handled.
 - All documentations on drug information queries shall be kept for a minimum of 5 years.

Legal and ethical issues related to DIS

- **Confidentiality**

- Enquirer's identity shall remain confidential.
- Patient information shall remain confidential.
- The substance of enquiries shall always remain confidential.
- Confidential industrial information shall be kept as such.

Legal and ethical issues related to DIS

- **Use of Social Media**

- If the DIS wishes to publish information using social media (such as Facebook, Twitter, Instagram or YouTube etc.), all legal and ethical principles and codes of conduct relating to pharmacy must be upheld.
- Patient and institution confidentiality must also be adhered to.
- Any material that is posted on social media should be publicly available and within the pharmacist's range of expertise.
- The service should be very cautious about or refrain from taking medicines information queries via social media due to the limitations in gathering all required data and risk of third party enquiries.

Legal and ethical issues related to DIS

Labeling and Advertising

- **Labeling:** Written or oral information used to supplement or explain a product, regardless of whether the information accompanies the product
 - Requires full disclosure
- **Advertisements**
 - Require a fair balance, meaning there must be a discussion of both benefits and risks, so as not to be misleading, and substantial evidence from clinical trials must be included for comparative claims

Legal and ethical issues related to DIS

Labeling and advertising liability

- Doctrine of over promotion, under which adequate warning is alleged to have been diluted by communications failing to adequately convey the full impact of the warning
- Promotion of off- label or non - FDA - approved indications

Legal and ethical issues related to DIS

e - Health technologies: Telemedicine and Cybermedicine

- **Telemedicine:** Use of telecommunications and interactive video technology to provide health care services to patients who are at a distance
- **Cybermedicine** is a broader concept that includes the marketing, relationship creation, advice, prescribing, and selling pharmaceuticals and devices in cyberspace
- **Telepharmacy** is a subset of telemedicine
- As telemedicine and Cybermedicine expand, questions regarding liability for pharmacists providing DI on the Internet will need to be addressed

Legal and ethical issues related to DIS

Copyright

- is a property right in an original work of authorship that is fixed in tangible form
- Pharmacists providing DI must have a working knowledge of copyright law, both to avoid liability and to protect their own literary works.
- Fair use of a copy right is determined by a four pronged test
 - i. Nature and character of use
 - ii. Nature of the work
 - iii. The proportional amount copied
 - iv. The effect on the market for the copied work

Defenses to Negligence and Malpractice

- Legal defenses can avoid or reduce liability
- Defenses include:
 - Statute of limitations
 - Comparative or contributory negligence
 - Informed consent
 - Governmental immunity
 - etc

Quality Assurance as a Liability- Reducing Factor

- Identify scope of activities and personnel requirements
- Develop and follow policies and procedures or formal call triaging protocols
- Keep standard operating procedure manual available for consultation
- Avoid violations of statutes and regulations
- Do not recommend an unapproved use or dose; if a use differs from the labeling, the requestor must be so notified
- Do not recommend a use or dose of a drug based solely on foreign literature or animal studies

Quality Assurance as a Liability- Reducing Factor

- Never extrapolate pediatric dosages from adult dosages
- Maintain knowledge of the current literature, new drug applications and supplemental approvals, labeling changes, and new warnings
- Do not present inadequate data or ignore contrary data
- Avoid overly enthusiastic or exaggerated efficacy and safety claims; do not attempt to diagnose or treat acute poisoning — direct such inquiries to a poison control center or an emergency room

Quality Assurance as a Liability- Reducing Factor

- Know the circumstances of the case and appropriate background information
- Responses of new employees, students, residents should be checked.
- Maintain reasonable response time; if necessary prioritize requests
- Obtain peer concurrence or outside professional consultation, if necessary
- Develop a QA mechanism to ensure that service is maintained at a high level of quality (e.g., periodic audits or surveys)

Thanks!